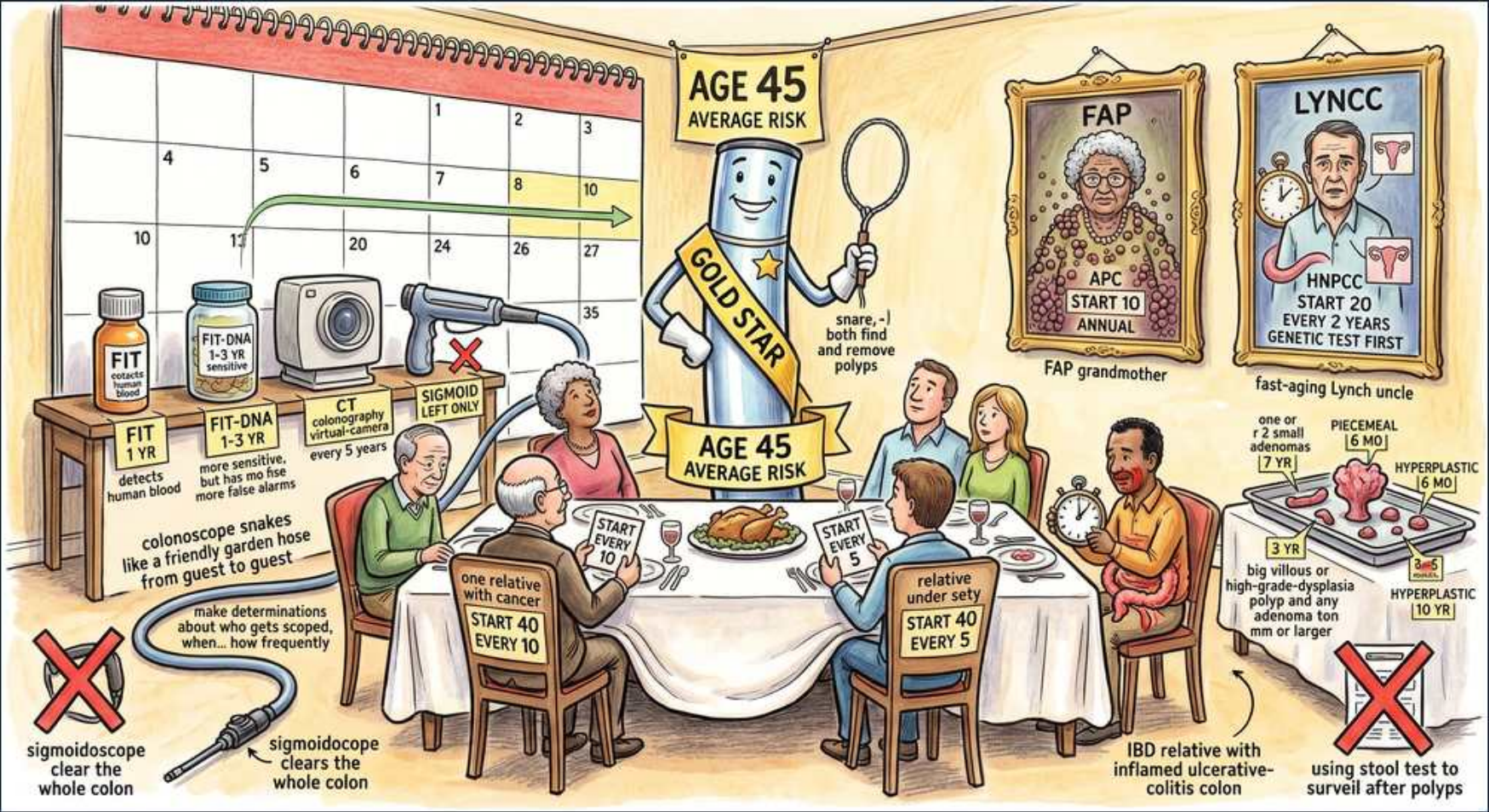


Endoscopy — CRC Screening & Surveillance



1. AGE 45 gold-star scope

WHAT YOU SEE

Scope character wearing a 'GOLD STAR' sash, 'AGE 45 AVERAGE RISK' banners top and bottom center.

WHAT IT ENCODES

GOLD STAR: average-risk screening now starts at age 45 (lowered from 50), colonoscopy every 10 years if normal. Continue through 75; individualize 76–85; stop at 85. Colonoscopy is the gold standard because it visualizes the entire colon and removes polyps in one step.

2. FIT yearly

WHAT YOU SEE

Bottle labeled 'FIT 1 YR — detects human blood', far left.

WHAT IT ENCODES

Fecal immunochemical test detects human globin, done yearly; any positive FIT mandates a full colonoscopy. Non-invasive but only a screen — does not remove polyps and misses lesions that aren't bleeding.

3. FIT-DNA 1–3 yr

WHAT YOU SEE

Washing-machine 'FIT-DNA 1-3 YR — more sensitive but more false alarms', left.

WHAT IT ENCODES

Multitarget stool DNA (Cologuard) every 1–3 years: higher sensitivity than FIT but lower specificity (more false positives). A positive result requires colonoscopy.

4. CT colonography 5 yr

WHAT YOU SEE

Camera labeled 'CT colonography virtual camera — every 5 years', left-center.

WHAT IT ENCODES

CT colonography every 5 years; non-invasive but requires bowel prep, and any significant finding still needs colonoscopy. Does not allow biopsy or polypectomy.

5. Sigmoidoscope LEFT ONLY (red X)

WHAT YOU SEE

Sigmoidoscope hose with red X, 'sigmoidoscope clears the whole colon' caption, lower left.

WHAT IT ENCODES

Flexible sigmoidoscopy reaches only the rectum and left colon (to splenic flexure) — it MISSES the right colon where many lesions and most serrated polyps occur. It is an acceptable screening modality but never 'clears the whole colon'.

BOARD TRAP

'A flexible sigmoidoscopy clears/screens the whole colon' → wrong; it only reaches the left colon and misses right-sided cancers.

6. FAP grandmother — APC, start 10

WHAT YOU SEE

Portrait 'FAP' grandmother, 'APC START 10 ANNUAL', upper right.

WHAT IT ENCODES

Familial adenomatous polyposis (autosomal dominant, APC gene, hundreds–thousands of polyps): annual sigmoidoscopy/colonoscopy from age ~10–12. Near-100% cancer risk untreated → prophylactic colectomy. Genetic testing of at-risk relatives.

7. Lynch uncle — HNPCC, start 20–25

WHAT YOU SEE

Portrait 'LYNCC' fast-aging uncle, 'HNPCC START 20 EVERY 2 YEARS GENETIC TEST FIRST', upper right.

WHAT IT ENCODES

Lynch/HNPCC (MMR genes MLH1/MSH2/MSH6/PMS2, microsatellite instability): colonoscopy every 1–2 years starting age 20–25 (or 2–5 yr before youngest case). Genetic testing first. Right-sided cancers, plus endometrial/ovarian/urothelial risk.

8. One relative with CRC — start 40, q5

WHAT YOU SEE

Diner with 'one relative with cancer — START 40 EVERY 10' and 'relative under 60 → START 40 EVERY 5' table signs, center.

WHAT IT ENCODES

One first-degree relative with CRC (or advanced adenoma) → begin at age 40, OR 10 years before the relative's age at diagnosis, whichever is earlier; repeat every 5 years (not 10).

9. 1–2 small tubular adenomas — 7–10 yr

WHAT YOU SEE

'one or 2 small adenomas → 3 YR / 7-10' note, right-center.

WHAT IT ENCODES

Post-polypectomy surveillance: 1–2 small (<10 mm) tubular adenomas, low-grade → repeat colonoscopy in 7–10 years. Normal-equivalent low-risk finding.

10. 3+ / advanced adenoma — 3 yr

WHAT YOU SEE

'3 YR — big villous or high-grade-dysplasia polyp and any adenoma 10 mm or larger', lower right.

WHAT IT ENCODES

High-risk polyps → repeat in 3 years: 3–10 adenomas, any adenoma ≥10 mm, villous histology, or high-grade dysplasia. Piecemeal resection of large sessile lesions or HGD warrants short-interval recheck at ~6 months.

11. Hyperplastic — 10 yr

WHAT YOU SEE

Red sessile polyp labeled 'HYPERPLASTIC 10 YR', far right.

WHAT IT ENCODES

Small distal hyperplastic polyps → 10-year interval (treated like a normal exam). Caveat: large (≥ 10 mm), proximal, or sessile serrated lesions carry malignant potential and follow adenoma-like shorter intervals.

12. IBD colitis surveillance — 8–10 yr

WHAT YOU SEE

Person captioned 'IBD relative with inflamed ulcerative-colitis colon', lower center-right.

WHAT IT ENCODES

Chronic colitis (ulcerative colitis or Crohn colitis) → start dysplasia surveillance colonoscopy 8–10 years after symptom onset, then every 1–3 years with chromoendoscopy / multiple biopsies. PSC + IBD → begin surveillance at diagnosis, annually.

13. Stool test for surveillance (red X)

WHAT YOU SEE

Big red X over 'using stool test to surveil after polyps', far lower right.

WHAT IT ENCODES

After a polyp is removed, surveillance must be COLONOSCOPY, not FIT/stool DNA. Stool tests only screen for bleeding lesions and cannot follow a known polyp-prone colon.

BOARD TRAP

'Use a yearly FIT to follow a patient after polypectomy' → wrong; post-polypectomy surveillance requires colonoscopy, not a stool test.